Business Contingency And Emergency Planning (v4.0.0)

Date Reviewed: 11/12/2024 | **Next Review Date:** 11/12/2025

Person responsible for updating this policy: Hitendrakumar Sharma

Policy Statement

Bluestar Care & Support- Bluestar Resourcing is aware of its responsibilities for the delivery of its commissioned services. This policy sets out the contingency arrangements that can be implemented when an unplanned critical or emergency event or force majeure situation arises.

A detailed business plan is in place.

Procedure

This policy aims to enable service delivery to take place even in unplanned situations. Working with multi-agency partners, Bluestar Care & Support- Bluestar Resourcing would seek to ensure the following measures were in place to minimise any disruption to planned services and to co-operate in any way possible to assist in any force majeure situation that may arise.

This policy sets out how Bluestar Care & Support- Bluestar Resourcing would seek to minimise the impact of unplanned situations.

The Business Continuity Plan

The Business Continuity Plan sets out the possible events that could disrupt the effective continuation of services. The plan sets out these events and the measures needed to mitigate their effects on the business. Plans must reflect the service and local conditions – for example, a plan in a very rural residential home will look quite different to that of a domiciliary service in a highly populated urban area.

The plan will typically set out;

- The events
- The effects of these events on the service user, staff or the business
- The probability of their occurrence – low, med, high
- The level of risk to the business/Service – low, med, high
- Mitigating action
- Names of those responsible for each action
- Timescales to rectify the disruption
- Communication methods during a disruption – verbal, written, email, when, to whom and how often
- Making statutory Notifications (See separate Notifications Policy).

As with any plan, it should be reviewed at least annually or after an actual event to assess its effectiveness. It is good practice to test elements of the plan at least annually by replicating one or more of the identified events and implementing the mitigating actions to test their effectiveness. This may identify weaknesses in the plan so that additional measures can be included to improve the response in the future and reduce the effects on the business.

Events that may trigger a Business Continuity response

- Outbreaks – such as acute respiratory infection
- Adverse Weather – snow, floods, high winds, heatwave

- Adverse Weather – snow, floods, high winds, heatwave
- Utility Failure – electric, gas, water
- Industrial Action – Strikes, overtime bans, public transport disruption
- Major events – concerts, sporting events, public demonstrations
- Major Roadworks – road closures
- Fuel Shortages
- IT Failure – failure of hardware, internet, cyber attack
- Terrorist Activity – bomb threats, targeted attacks
- Civil unrest – riots, blockades
- Financial – lack of funds to continue operating the business
- Failure of essential suppliers.

Example of Mitigating Actions Utility Failure in the Office

- staff can work from home
- Software systems are cloud-based and can be accessed from any location
- Office staff issued with company laptops
- Phones diverted to Mobile devices
- Sign out sign-in procedure for hard copy information removed from the office
- Alternative storage of PPE
- Regular communication with staff, the service user and wider stakeholders on the status and likely duration of the disruption.

Communicating the Plan

All staff must have access to the plan and understand its importance and the role it will play during a disruptive event. The plan will be communicated via staff meetings, supervision and appraisal sessions and a copy will be displayed in the office at all times.

Bluestar Care & Support- Bluestar Resourcing along with local authorities can access a Business Continuity Plan Audit Tool this aims to consider how thoroughly cyber security and data protection issues are addressed as part of our business continuity plan. It will enable Bluestar Care & Support- Bluestar Resourcing and local authorities to understand how assured we can feel about the level of planning in place to minimise the impact of a data breach or cyber attack, we will also use the Data Security & Protection Toolkit at least once a year to check and improve our data security arrangements. We use the tool to check how thorough our business continuity plans are, and whether these require any amendments.

Late Visits

This is different from missed visits, which is a separate policy, and the two should not be confused. A late visit is when the scheduled visit time is not met by the assigned staff. There are usually built-in lateness periods via Local Authority (LA) service specifications, and these are usually between 15 and 30 minutes duration. For example; a scheduled visit at 7.30 pm would not be considered a late visit until 7.45 pm or 8 pm, using the agreed lateness duration of the LAs we work with. Our policy is to have a maximum lateness time of [insert maximum lateness time in minutes/hours] when attending to service users. .

Late visits can be caused by many different situations, e.g. a medical emergency, such as a fall or stroke, where medical assistance is needed. The scheduled visits on that round would need to be covered, as it is likely the staff would be delayed until the arrival of the emergency services. Family, friends, or representatives will be kept informed.

Utility Failure

From time to time, a utility failure occurs that impacts the home of a service user, We would be able to access camping gas and water to enable the service user to be cared for in terms of personal care, warmth, and nutritional needs. We would keep in contact with the family (where applicable), the utility agency (to ensure we could respond appropriately) and, where a large section of the community was affected, the relevant statutory agencies (e.g. police, social services) and the emergency civil planning department of the Local Authority (LA) where necessary.

Adverse Weather/ Winter Planning

As part of contingency planning, we prepare for adverse weather events and understand the impacts these have on health and well-being. We understand the actions that need to be taken individually and organisationally to ensure the safety and health of every service user and staff during such events and the preparations to be taken in advance.

We ensure that all staff working with the service user is trained to understand the impact of adverse weather events – such as extreme heat and cold – on bodily functions and to be alert to the physical and mental signs that impact on health. All staff are made aware of the Weather-Health Alerting service and the good practice actions.

We regularly monitor the UK Health Security Agency (UKHSA) guidance on the government website and update our procedures as necessary.

When adverse weather is predicted information is relayed to every service user, their carers and family as soon as possible in accessible formats such as word of mouth, emails and Easy-read.

We work closely with our staff, to support them during work time to travel safely in adverse weather conditions. As an employer, we encourage staff to take warm clothing with them in winter or adequate fluids with them in summer in the event of a breakdown, in any circumstance.

Risk assessments are in place to identify what is reasonably foreseeable and practical to provide when driving is required as part of their working day. This includes Limb A and B workers (The Personal Protective Equipment at Work (Amended) Regulations 2022 (PPER 2022)) should the temperature be likely to fall below the level stipulated in the Workplace (Health, Safety and Welfare) Regulations 1992.

Such situations would require rescheduling the visits. Families and social services would be contacted and informed, and an explanation is given as to why the changes had been implemented.

To minimise travel, Care and Support staff would be scheduled to start as close to home as possible and some could be scheduled to walk, where flooding, snow, or ice was present. The employment of locally-based staffwould assist in this situation.

The involvement of family and neighbours would be considered for any service user whose needs could be met by this assistance. Every service user would be contacted and given information and advice pertinent to the service user, e.g. the times of visits and who would be making them.

A statutory notification must be sent to the Care Quality Commission (CQC) if the adverse weather was likely to last more than 24 hours.

The impact of any outbreak on our health and social care system is ongoing and is why winter and surge planning is now a continuous and integral part of our work, with work on surge planning and delivery.

We are implementing best practices to deliver system improvements and build capability including the development of contingency options for winter months.

This means we are actively monitoring and evaluating strategic risks and system pressures to allow timely decision-making that is closely coordinated with Health Boards and and Care Quality Commission (CQC) revising our business contingency and emergency planning as necessary.

Respiratory conditions are always a concern during the winter months, this can make it even more challenging when caring for the service user

The service user is at the heart of what we do, and to guide and focus our collective efforts in preparing for winter we will:

- Where clinically appropriate, ensure people receive Care and Support at home, or as close to home as possible, promoting access to the right care, in the right place, at the right time
- Focus on expanding our workforce over the course of the winter, through recruitment, retention, and well-being of our staff, all to expand and support our workforce
- Continue to deliver as safe care as possible throughout the autumn/winter period
- Work in partnership across health and social care to continue to deliver safe care through the winter months.

Adverse weather- Service Users

Service Manager Hitendrakumar Sharma and staff do whatever they can to identify any service user who may be at risk in cold weather and put into place measures to reduce risks. staff who visit people at home should assess the heating needs of the service user and inform the office of any problems. With their consent, they can be referred to the local housing support services or other local charities that can help.

When a service user is identified as being at risk, during cold weather, our Care and Support managers and staff consider what can be done to provide extra support. This may include additional visits to check they are keeping warm and have hot food and drink.

These considerations should also extend to carers – considering their needs and what support they can access in adverse cold weather.

If staff are unsure whether someone is at risk advice will be sought from from their GP or other health professionals, with the consent of the service user.

We develop and maintain a register of any service user who is at risk in adverse weather and prioritise their care and support accordingly.

In cold weather, staff should advise the service user to:

- Keep their heating turned on
- Seek advice from utility companies or charities if worried about fuel costs.
- Never cover heaters and fires, such as with drying clothes
- Never block air vents if they have wood-burning, coal or gas heaters
- Advise them to check that their carbon monoxide and fire alarms are in good working order
- Draw curtains and keep bedroom windows closed at night
- Wear more clothes at night such as bedsocks and thermal underwear.
- Dress in plenty of layers of warm clothing
- Eat healthily and have regular hot drinks
- Keep a list of emergency numbers nearby
- Ask friends or family to call in
- Where necessary, our workers might also liaise with a vulnerable person's GP or with local charities and community support groups who distribute essential items to help keep people warm such as clothes, heaters and electric blankets
- For people who rely on food banks, some charities will deliver food where individuals at risk are unable to get out of their homes because of the cold

Sustained periods of cold and icy weather can disrupt travel and affect the ability of Care and Support staff to get to the service user. In such cases, we consider alternative ways of providing care that can be put in place, including liaising with neighbours.

We prioritise the most vulnerable service user to ensure they are supported and visited throughout any severe weather period.

Adverse Weather - Summer Planning

Refer to the Heatwave policy for precautions and procedures.

Temperature in the workplace (Office Based)

The Workplace (Health, Safety and Welfare) Regulations 1992 require employers to provide a reasonable indoor temperature in the workplace.

This depends on the work activity and the environmental conditions.

The Approved Code of Practice on the Workplace (Health, Safety and Welfare) Regulations 1992 states the minimum temperature for working indoors should normally be at least:

- 16°C or
- 13°C if much of the work involves rigorous physical effort

Bluestar Care & Support- Bluestar Resourcing provide:

- A reasonable working temperature for our office-based staff – at least 16°C
- Local heating or cooling (using fans, opening windows, using radiators) where a comfortable temperature cannot be maintained throughout each workroom
- Rest facilities where necessary
- Heating systems which do not give off dangerous or offensive levels of fume into the workplace

Bluestar Care & Support- Bluestar Resourcing takes these practical steps to keep staff as comfortable as possible:

- Provide adequate workplace heating, such as portable heaters, to ensure office areas are warm enough when they are occupied
- Reduce draughts while still keeping adequate ventilation
- Limit exposure by introducing systems such as flexible working patterns or job rotation where appropriate and possible
- Provide enough breaks to allow staff to get hot or cold drinks.

Outbreak Management

UK Health Security Agency (UKHSA) and Office for Health Improvement and Disparities now has the responsibility to protect public health from such an outbreak and to provide guidance to organisations where the impact of such a outbreak could be catastrophic. They regularly publish preparedness strategies and response plans, etc., in the event of such a situation.

The five phases of detection, assessment, treatment, escalation, and recovery are monitored, appropriate data collection, the route of the pandemic tracked, and advice and guidance issued, as appropriate.

Acute Respiratory Infections including COVID-19

Homecare providers must ensure that the level of Care and Support support provided to a service user meets their assessed needs. However, home care providers may need to reallocate duties or reduce visits if a service user being cared for has a respiratory infection including acute respiratory infections. These contingency plans will need to:

- Be made subject to agreement with partner agencies and/or commissioners that the reduction in duties or visits balances the risks of reducing care with that of potential transmission of the respiratory infection
- Consider the Care and Support needs, wishes, and feelings of the relevant person and the unpaid carer or carers, in line with a personalised care approach
- Where significant changes are made, be based on a new assessment of the needs of theservice user - the new level of support provided to the service user must meet their assessed needs
- Be made in agreement with staff and family with the involvement and consent of the service user.

Providers should maintain business continuity plans to help manage emergencies. These should be kept up-to-date and key details to record may include:

- Who provides Care and Support for the service user
- Whether those delivering care are still able to provide care and are not self-isolating, whether paid staff or informal carers
- How and where Care and Support plans are located
- Requirements for any specialist care or long-term conditions/ people formally identified as clinically extremely at risk
- Modes of communication
- Key contacts coordinating care from other community-based services including, but not limited to:
    - mental health and dementia support services
    - learning disability services
    - third-sector voluntary social and community enterprises (VSCEs)
    - drug and alcohol or social work teams
    - family members.

It is particularly important to ensure risk management plans are balanced and updated for individuals who may find any change in routine challenging, for example, people living with dementia and certain types of autism. This should include preparation for and likely reactions by the service user to changes in routine or unfamiliar carers, and ways to reduce potential stress. In cases where current circumstances make consistency impossible, providers should prepare people for the fact that it may be necessary for a different carer to support them.

Providers and local authorities should work together to facilitate mutual aid, and Care and Support plans across their areas. This is to inform planning ahead of a possible outbreak.

Routine movement of Care and Support staff between any shared living services and other health and social care settings should be avoided to reduce the potential spread of acute respiratory infections including COVID-19 from one setting to another following current government guidance.

By following these steps, most people we support should have a continuity of care, support, and help that adapts to their situation.

For people, where well-being is at risk, Service Manager Hitendrakumar Sharma may need to contact individual social services staff to seek further advice and support.

Health and Care Act 2022

The Act introduces measures to tackle the acute respiratory infection backlogs and rebuild health and social care services from the pandemic, backed by £36 billion over the next 3 years through the Health and Care Levy. It will also contain measures to tackle health disparities and create safer, more joined-up services that will put the health and care system on a more sustainable footing.

To continue to safeguard people receiving our services, in relation to the COVID-19 pandemic, the Health and Care Act

To continue to safeguard people receiving our services, in relation to the COVID-19 pandemic, the Health and Care Act 2022 (Adult social care provider information provisions: guidance for providers on data collection) requires us to submit a core subset of the data which has previously been submitted through the Capacity Tracker (CT). This is gathered on a monthly basis. We are required to update data by the end of the 14th day of each month, or the next working day where the 14th falls on a weekend or public holiday. Data must be no more than a week out of date - that is, data must be correct to no further back than the 8th of each month.

As a domiciliary service regulated by the Care Quality Commission (CQC) , the mandatory information required is:

- How many service users are currently registered and receiving care from our agency
- Number of staff in the organisation that have face-to-face contact with the people being supported
- Number of staff delivering care that are not working because of COVID-19
- How many hours we have paid direct employees to work in the last month or 4 weeks, including any overtime
- How many hours of overtime we have paid for in the last month or 4 weeks
- How many hours of agency staff time we have paid for in the last month or 4 weeks
- How many days or hours of absence we have recorded in the last month or 4 weeks
- COVID-19 vaccination (during a campaign and up to the mandation week following the end of a campaign):
- number of staff known to have received a seasonal booster
- Flu vaccination (seasonal: 1 September to 31 March):
- number of staff known to have received this season's flu vaccination
- Packages of care:
- In the last 28 days, how many current NHS and local authority-funded service users' care packages we have had to return
- In the last 28 days, how many current self-funded service users' care packages we have had to end.

Staffing

Staffing is the biggest issue for the continuity of service to be ongoing and, when necessary, statutory notifications should be completed to assist with the planning required for cover.

All LAs have an Emergency Civil Plan (ECP), Civil Emergency Plan (CEP), or Civil Contingency Plans (CCP) that is activated when certain criteria are met. Our local ECP, CEP, CCP (DELETE WHICH DO ONES DO NOT APPLY] can be accessed via [DETAIL WHERE THIS IS LOCATED]

A multi-agency approach is in place via the health authority and UK Health Security Agency (UKHSA) and Office for Health Improvement and Disparities, and Bluestar Care & Support- Bluestar Resourcing will follow all available advice and guidance in managing any pandemic or similar situation. staff will be advised as to their actions via the office.

Force Majeure Situations

Where a force majeure was in place, e.g. major flooding, fuel shortages, road closures, and winter conditions, we would take advice and co-operate in any way possible with the civil emergency team and the statutory agencies involved. This could include:

- Emergency centres being utilised
- Evacuation procedure
- staff secondment to assist
- Assisting other providers with available beds.

We have good local knowledge, and our relationship with our multi-agency partners would enable us to deliver the service, except where advice was given to the contrary. We are aware of winter plans from West Sussex County Council and the NHS and would seek appropriate advice immediately to manage the situation effectively**.** We have good local knowledge, and our relationship with our multi-agency partners would enable us to deliver the service, except where advice was given to the contrary. We are aware of winter plans from our LA and the NHS and would seek appropriate advice immediately to manage the situation effectively.

Industrial Action - We are aware of national and local industrial action and the potential impact it may have on a service user in an emergency situation.

A statutory notification must be sent to Care Quality Commission (CQC) if any of the above situations were likely to last more than 24 hours.

Civil Unrest

During times of high tension, we ensure our office premises are secure. We stay informed and updated with the latest

news through reliable sources. Our priorities are to keep every service user and the whole workforce safe and supported during these times of unrest.

As employers, we have a duty of care to protect our staff health, safety and welfare. We therefore take the necessary steps to ensure the workplace is safe for all staff and ensure that the workplace is free from discrimination and harassment.

To prepare for and during periods of civil unrest and riots we regularly monitor and review our business contingency plan by:

- Reviewing our risk registers to ensure civil unrest and associated risks are included and regularly evaluated
- Conducting a risk assessment to identify potential threats to staff
- Identifying staff who may be at higher risk for example due to ethnicity, beliefs or commuting routes and developing personalised safety plans with these at-risk staff
- Keeping staff informed about the current situation and safety measures via regular updates
- Establishing a point of contact for staff to report concerns or seek assistance
- Offering flexible working hours or remote working options to avoid travel during unrest where possible and appropriate
- Providing safe transportation alternatives, such as arranging travel buddies, covering taxi fares, or offering parking spaces
- Reviewing our mental health resources including training to enable managers and first aiders to recognise and support staff experiencing trauma or stress
- Actively monitor local media coverage, local authority alerts and information, responding to ensure people's safety
- Updating our business continuity plans to include strategies to cope with staff shortages, and any other contingency plans, and ensure these are shared and understood by everyone in Bluestar Care & Support- Bluestar Resourcing and other stakeholders
- Reviewing our insurance provision to understand what is and is not covered and where the gaps may need addressing
- Communicating regularly with the service user and their family and friends, updating them with any necessary steps we need to take to ensure their safety
- Notify the Care Quality Commission (CQC) of any disruption to our service.

Addressing staff involvement in riots - Legal and Disciplinary Actions:

- We consult with legal advisors to understand the implications and ensure compliance with employment and criminal law
- We follow company policies for disciplinary actions, ensuring a fair and thorough process
- We follow our Adult Safeguarding procedures
- We send safeguarding notifications to West Sussex Safeguarding Unit and Bluestar Care & Support- Bluestar Resourcing as required .

Bluestar Care & Support- Bluestar Resourcing have a critical role in supporting our staff during times of unrest. We navigate these challenges effectively and maintain a resilient and united workforce by prioritising safety, providing mental health support, handling allegations sensitively, and promoting an inclusive culture.

Our independent HR professionals support us by providing the expertise, guidance, and personalised support needed to achieve these goals.

Procedures for Handling Bomb Threats

All bomb threat communications are a crime and should be reported to Service Manager Hitendrakumar Sharma or person in charge who will immediately contact the police by dialling 999.

While many bomb threats involve a person-to-person phone call, an increasing number are sent electronically using email or social media. No matter how ridiculous or implausible the threat may seem, all such communications are a crime and must be reported to the police by dialling 999.

The bomb threat message

Bomb threats containing accurate and precise information, and received well in advance of an actual attack, are rare occurrences. Precise motives for hoaxing are difficult to determine but may include revenge, extortion, a desire to impress, or a combination of these and other less understandable motives. Hoaxes are a form of social engineering and, whatever the motive, the intent is to cause disruption, fear and/or inconvenience the victim, i.e. to make victims work against their own best interests.
Communication of the threat

Communication of the threat

A bomb threat can be communicated in many different ways. The threat is likely to be made in person over the telephone however it may also be:

- A recorded message (possibly using a text-to-speech synthesiser or a soundboard)
- Communicated in written form
- Delivered face-to-face
- Sent by email or social media
- A threat may be communicated via an independent third-party, i.e. a person or organisation unrelated to the intended victim and selected only to pass the message. However the threat is communicated, the police will need the exact wording and whatever other details are available.

Procedure

Immediate steps if you receive a bomb threat communication.

Any staff member with a direct telephone line, mobile phone, computer or tablet etc. or who has any contact with the public, could receive a bomb threat.

Our staff must, therefore, understand the actions required of them as the potential first response to a threat message and their duty of care to others.

The Bomb threats checklist identifies actions to be taken and prompts questions to be asked If a telephone threat is received:

- Stay calm and listen carefully
- Have immediate access to the bomb threat checklist and the key information that should be recorded
- If practical, keep the caller talking and alert a colleague to dial 999
- If displayed on your phone, note the number of the caller, otherwise, dial 1471 to obtain the number once the call has ended
- If the threat is a recorded message, write down as much detail as possible and retain it for the police to secure
- If the threat is received via text message, do not reply to, forward or delete the message; note the number of the sender and follow police advice
- Know who to contact in Bluestar Care & Support- Bluestar Resourcing upon receipt of the threat, e.g. building security and senior manager, as they will need to contact the police and direct any evacuation. in our organisation this contact is: [Hitendrakumar Sharma/ Director AND 07809721605]

If the threat is delivered face-to-face:

- Try to remember as many distinguishing characteristics of the person who made the threat as possible and try to remember exactly what was said.

If discovered in a written note, letter or as graffiti:

- Treat as police evidence and stop other people from touching the item.

If the threat is received via email or a social media application:

- Do not reply to, forward or delete the message
- Note the sender's email address or username/user ID for social media applications
- Preserve all web log files for your organisation to help the police investigation (as a guide, police will require data from 7 days before the threat message and 48 hours after).

There may be circumstances when staff working in the home of a service user experience a bomb threat for that location or area.

staff must keep in touch with the office, follow any advice and implement evacuation guidance from the police or other emergency services.
Service Manager Hitendrakumar Sharma will keep updated with any changes in the situation and any updates will be relayed as soon as possible to staff and family members.

The office will inform the staff member when the bomb threat is over and it is safe to leave. They will get this information from the emergency services.

All incidents will be recorded on an incident form.

A notification must be sent to the Care Quality Commission (CQC).

- **Immediate Evacuation:** Leave the building quickly, following evacuation routes. Do not stop for belongings.
- **Suspicious Items:** Do not touch any suspicious packages or devices. Report them to security or law enforcement.
- **Notify Authorities:** Call 911 immediately. Provide location and details of the threat.
- **Follow Instructions:** Obey instructions from law enforcement officials.
- **Safety Measures:** Do not use elevators or phones during evacuation.
- **Accountability:** Take a headcount to ensure everyone has evacuated.
- **Safe Assembly Area:** Establish a safe assembly area away from the building.
- **Return:** Do not return to the building until authorized by law enforcement.

Training Statement

All staff, during induction, are made aware of the organisation's policies and procedures, all of which are used for training updates. All policies and procedures are reviewed and amended where necessary and staff are made aware of any changes. Observations are undertaken to check skills and competencies. Various methods of training are used including one to one, online, workbook, group meetings, individual supervision and external courses are sourced as required.

Related Policies

- Continuity Of Care For Support Workers (Domiciliary)
- Co-operating With Other Providers (Domiciliary)
- Duty Of Candour (Domiciliary)
- Recruitment And Selection (Domiciliary)
- Sponsorship Licence For Employment of Staff From Overseas (Domiciliary)

Related Guidance

- GOV.UK: Preparing for emergencies: find out about local plans
- NHS England: Winter resilience
- HSE: Temperature in the Workplace
- GOV.UK: Guidance: Heatwave Plan for England
- GOV.UK: Supporting vulnerable people before and during a heatwave: for health and social care professionals
- GOV.UK: Infection prevention and control in adult social care settings
- GOV.UK: Health and Social Care Worker Visa
- HSE: Approved Code of Practice
- CQC: Regulation 17 - Good Governance
- GOV.UK: Health and Care Act 2022
- UK Parliament: Changes to legal migration rules for family and work visas in 2024
- Department of Health and Social Care: Infection Prevention and Control
- Digital Social Care: Business Continuity Plan Audit Tool
- Digital Care Hub: DSPT
- UK Health Security Agency: Adverse Weather and Health Plan
- Digital Care Hub The Digital Switchover: From Analogue to Digital

Policy Legal Statement

This policy is Copyright © W&P Assessment & Training Centre Ltd. 2024 and is only licensed for use with a current W&P Portal Contract. If you have a current contract, it can be accessed in your online account. Use of this policy without a current W&P Portal Contract is strictly prohibited.